

Indication	**Antibiotic drug	Regimen	Default Duration
	Approval by ID required for linezolid if continued beyond 72 hours. Micafungin requires ID approval.	NA	
	1. preferred therapy 2. severe Penicillin allergy (anaphylaxis)	(a) monitor; adjust if renal dysfxn (b) kinetic monitoring	
Pneumonia			
Community acquired (CAP)	1. ceftriaxone + azithromycin	2 g IV q24h 500 mg PO/IV q24	5 days 3 days
	2. levofloxacin	(a) 750mg IV q24 hours	5 days
early (< 5dy) hospital acquired (HAP), healthcare associated (HCAP), ventilator associated (VAP), or aspiration pneumonia (not chemical pneumonitis)	1. ceftriaxone	2 g IV q24h	5 days
	2. aztreonam + vancomycin	(a) 2 g IV q8h (a,b) see nomogram	5 days
late (≥5dy) or risk factors for multi-drug resistant (MDR) pathogen HAP, VAP, or aspiration pneumonia (not chemical pneumonitis)	1. cefepime + vancomycin	(a) 2 g IV q8h (a,b) see nomogram	5 days then reassess – if WBC and fever curve have not normalized, then 7 days
	2. aztreonam + vancomycin	(a) 2 g IV q8h (a,b) see nomogram	5 days then reassess – if WBC and fever curve have not normalized, then 7 days
Urinary tract infections			
urinary catheter; UTI	1. ceftriaxone	1 g IV q24h	Change and/or remove catheter if possible followed by 4 days of therapy
	2. aztreonam OR 3. aminoglycoside	(a) 2 g IV q8h (a,b) Gentamicin/Tobramycin 3mg/kg IV x 1 dose Amikacin 5-7mg/kg x 1 dose	
Urinary catheter;UTI;candida	Fluconazole	(a) 400mg IV/PO once followed by 200mg IV/PO x1	2 days (load and follow up)
Bacteremia			
	1. cefepime + vancomycin	(a) 2 g IV q8h (a,b) see nomogram	7 days*
	2. aztreonam + vancomycin	(a) 2 g IV q8h (a,b) see nomogram	7 days*
Wound / Soft tissue			
necrotizing soft tissue infection; necrotizing fasciitis	1. linezolid + pip/tazo	600mg IV q12h (a) 4.5g IV q6h	Until wound is clean
	Add doxycycline if suspected Vibrio exposure	100mg IV q12h	--linezolid continued past 72 hours requires ID approval
	2. linezolid + aztreonam + metronidazole	600mg IV q12h (a) 2g IV q8h 500mg IV q8h	
	Add doxycycline if suspected Vibrio exposure	100mg IV q12h	

Intra-abdominal				
Pseudomonas low risk		1. ceftriaxone + metronidazole	2 g IV q24h 500mg IV q8h	4 days after source control achieved
		2. aztreonam + metronidazole + vancomycin	(a) 2 g IV q8h 500mg IV q8h (a,b) see nomogram	
Pseudomonas high risk (previous hosp or broad spectrum antibiotic exposure; (+) Pseudomonas culture result		1. cefepime + Metronidazole OR Pip/tazo --can add vancomycin if MRSA risk factors are present	(a) 2g IV q8h 500mg IV q8h (a) 3.375g q6h (a,b) see nomogram	4 days after source control achieved
		2. aztreonam + metronidazole + vancomycin	(a) 2 g IV q8h 500mg IV q8h (a,b) see nomogram	
		2. aztreonam + Clindamycin	2 g IV 600-900 mg	
Penetrating abdominal trauma Hollow viscous injury		1. ceftriaxone + Metronidazole	2g IV q24h 500mg IV q8h	24 hours
		2. aztreonam + Clindamycin	(a) 2 g IV q8h 600-900 mg q8h	
Candidemia				
Candida albicans/dubliniensis, tropicalis, parapsilosis, lusitaniae		Fluconazole	(a) 12mg/kg IV x1, then 6mg/kg IV q24h	14 days after first negative culture
Candida glabrata		Consider micafungin empirically Can switch to fluconazole if susceptible	100mg IV q24h (a) 12 mg/kg IV q24h	14 days after first negative culture
Clostridium difficile				
Initial episode, non-severe		Oral vancomycin	125mg PO q6h	10 days
Initial episode, severe		Oral vancomycin	125mg PO q6h	
Initial episode, fulminant		Oral vancomycin + Metronidazole --can consider vancomycin enemas if ileus is present	500mg PO q6h 500 mg IV q8h 500mg RE q6h	10 days (if no concomitant antimicrobials)
Immediate post-colectomy		Metronidazole --can consider vancomycin enemas at the discretion of surgeon	500mg IV q8h	5 days
Fractures				
Long bone (open) ***Duration from skin closure***	Grade I or II	1. cefazolin	(a) 2 g x 1 then 1g IV q8h (2g IV q8h for weight > 120 kg)	2 days
		2. clindamycin	900 mg IV q8h	2 days
	Grade III	1. ceftriaxone	2 g IV q24h	2 days

		2. clindamycin	900 mg IV q8h	2 days
	Fracture associated with farm environment, extensive severe soft tissue crush injury, or vascular compromise	1. Add Penicillin G	(a) 4 million units IV q4h	2 days
		2. Add clindamycin	900 mg IV q8h	2 days
	Presence of water contamination	1. Piperacillin/tazobactam + doxycycline	(a) 4.5g IV q6h, 100mg IV q12h	2 days
2. Aztreonam + metronidazole + doxycycline		(a) 2g IV q8h, 500mg IV q8h, 100mg IV q12h	2 days	
Facial	Orbital, sinus, midface	1. Cefazolin or ampicillin/sulbactam (if cefazolin is on shortage)	(a) 2g IV q8h, 3g IV q6h	24 hours upon admission
		2. Clindamycin	600 mg IV q8h	24 hours upon admission
	Non-operative mandibular	1. Cefazolin or ampicillin/sulbactam (if cefazolin is on shortage)	(a) 2g IV q8h, 3g IV q6h 600 mg IV q8h	24 hours upon admission
		2. Clindamycin	600mg IV q8h	24 hours upon admission
	Operative mandibular	1. Cefazolin or ampicillin/sulbactam (if cefazolin is on shortage)	(a) 2g IV q8h, 3g IV q6h 600 mg IV q8h	Admission through 24 hours after procedure
		2. Clindamycin	600mg IV q8h	Admission through 24 hours after procedure
ENT				
External flaps, non-radiated	Topical abx: Cleanse incision line with NS and apply bacitracin afterwards TID x 2 weeks	1. ampicillin/sulbactam 2. clindamycin	(a) 1.5g IV q6h 600mg IV q6h	24 hours
Oral and Pharyngeal flaps, non-radiated, not infected	Cleanse neck incision line with NS and apply bacitracin afterwards TID x2 weeks Peridex oral swabs QID	1. ampicillin/sulbactam 2. levofloxacin + metronidazole	(a) 1.5g IV q6h (a) 500mg IV q24h 500mg IV q8h	24 hours
External flaps or Oral and Pharyngeal flaps + radiated or infected	Cleanse neck incision line with NS and apply bacitracin afterwards TID x2 weeks Peridex oral swabs QID	1. ampicillin/sulbactam OR cefazolin + metronidazole 2. levofloxacin + metronidazole	(a) 1.5g IV q6h OR (a) 1g IV q8h, 500mg IV q8h (a) 500mg IV q24h 500mg IV q8h	72 hours
	Pseudomonas high risk	1. cefepime + metronidazole 2. levofloxacin + metronidazole	(a) 1g IV 8h, 500mg IV/PO q8h (a) 500mg IV q24h 500mg IV/PO q8h	
Radionecrosis	Cleanse neck incision line with NS and apply bacitracin afterwards TID x2 weeks Peridex oral swabs QID	1. ampicillin/sulbactam OR cefazolin + metronidazole 2. levofloxacin + metronidazole	(a) 1.5g IV q6h OR (a) 1g IV q8h, 500mg IV/PO q8h (a) 500mg IV q24h 500mg IV/PO q8h	7 days
	Pseudomonas high risk	1. cefepime + metronidazole 2. levofloxacin + metronidazole	(a) 1g IV q8h, 500mg IV q8h (a) 500mg IV q24h, 500mg IV/PO q8h	

* Staphylococcus aureus = 14 days after first negative blood culture, Candida = 14 days after the first negative blood culture, Enterobacteriales = 7 days, Enterococcus = 7 days, Pseudomonas = 7 days

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AmpC β -lactamase-producing *Enterobacterales* such as *Hafnia alvei*, *Enterobacter cloacae*, *Citrobacter freundii*, *Klebsiella aerogenes*, *Yersinia enterocolitica*, have clinically significant *ampC* expression where treatment with ceftriaxone and ceftazidime should be avoided, even if an isolate initially tests susceptible to these agents.